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June 16, 2025

The Honorable Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services
200 Independence Avenue, S.W.
Washington, D.C. 20201

Submitted electronically via http://www.regulations.gov

Dear Secretary Kennedy:

As the leading authority on oral health in the United States, the American Dental Association (ADA), representing over 159,000 dentists across the country, writes to present our comments on the publication in the May 18, 2025, edition of the Federal Register entitled "Health Technology Ecosystem Request for Information" released by the Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology Policy, Office of the National Coordinator for Health Information Technology (ASTP/ONC), Department of Health and Human Services (HHS).

We appreciate the HHS, CMS, and the ASTP/ONC's continued emphasis on interoperability, which facilitates the exchange of information between providers, their patients, and payers. This enables the effective and responsible adoption of technology, which empowers patients and aids them in making better decisions about their health and well-being.

The ADA comments ask that to achieve the objective of optimal health, **which includes oral health**, meeting dental providers and practices where they are and supporting them to ensure meaningful participation in the healthcare system is essential. Electronic dental records (EDRs) differ from electronic medical records (EMRs) in structure and use. EDRs are typically procedure-based. Structured dental data is generally less comprehensive and standardized than medical data. Additionally, most dental practices are small, private offices with limited information technology (IT) resources compared to larger group practices, federally qualified health centers (FQHCs), or hospital-affiliated settings. This variation in practice models presents a significant challenge for the dental health technology ecosystem.

Dental Health Information Technology Ecosystem – General Comments

Dental Specific Data Considerations

Electronic dental records are used differently and contain different information from electronic
medical records. Most electronic dental record systems were designed to capture visit-based

documentation of oral health conditions and interventions. For example, data captured is specific to oral anatomy, specific to the tooth, tooth surface, or probing site[1]. They may include measurements or observations, but diagnostic coding does not typically capture this information. Although standardized code systems exist[2], they have not been widely used in dentistry. Further, procedure-based dental records may not include the necessary granular details about materials, methods, or other considerations such as duration and complexity. **Despite these differences, we believe that dental-dental and dental-medical interoperability is foundational to achieving optimal health and should be pursued in a manner that accounts for differences in systems and needs of dental patients and providers.**

For example, the ADA has been spearheading the establishment of code sets, terminologies, and data exchange standards to continue to encourage interoperability. The ADA's ANSI/ADA Standard No.1084 - Reference Core Data set for communication serves as the basis for defining dental-specific data elements in the HL7 FHIR® Implementation Guide: Dental Data Exchange, Release 1 - US Realm.

In the July 2024 Standards Bulletin, in conjunction with United States Core Data for Interoperability (USCDI) Version 5, the ASTP/ONC included the Code on Dental Procedures and Nomenclatures (CDT) in the Corrected Applicable Standard Reference for Procedures. The ADA appreciates the ASTP/ONC's efforts to ensure that CDT maintains parity with other standard terminologies in the procedures data element. The ADA was particularly encouraged by the publication of USCDI Version 5 and stands ready to assist USCDI in the continued development and maintenance of its data elements.

Dental Imaging

Dentists use radiographic imaging to help diagnose and effectively treat dental disease. The ADA has worked with the Food and Drug Administration (FDA) to publish the ADA / FDA Guide to Patient Selection for Dental Radiographs.[3] These guidelines assist dentists in using diagnostic imaging based on their judgment. While radiographs help diagnose oral diseases, dentists must weigh the benefits against the risks of cumulative x-ray exposure. With their understanding of the patient's health history and disease susceptibility, dentists are best suited to make these decisions.

In collaboration with industry stakeholders and imaging vendors, the ADA developed standards for imaging in dentistry that are universally applicable to any digital imaging system, providing a scientifically validated process from image capture to image display to achieve an optimal balance between image quality and radiation dose[4][5]. The ADA has

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[1] ISO/ANSI/ADA Standard No. 3950:2010 (R2015) - Dentistry — Designation system for teeth and areas of the oral cavity

[2] ANSI/ADA Standard No. No. 2000.8-2025 -- Systematized Nomenclature of Dentistry (SNODENT)

[3] U.S. Food and Drug Administration. (n.d.). The selection of patients for dental radiographic examinations. In Medical X-ray Imaging. U.S. Department of Health and Human Services.

[4] ADA Technical Report No. 1057 — Interoperability of Imaging Technologies in Today's Dental Practice: 2010 (Reaffirmed 2023).

[5] ADA Technical Report No. 1048 Attachment of DICOM® Datasets Using E-Mail in Dentistry.

formally recognized the Digital Imaging and Communications in Medicine (DICOM®), which provides guidelines for the formatting, storing, and exchanging digital radiography files to ensure they are interoperable across all types of imaging systems, practice management systems, and health care networks. By conforming to DICOM®, dentists can ensure their digital radiography systems continue to be interoperable in the future instead of being tied to an individual vendor or proprietary file format that may become obsolete.

However, the failure of imaging systems to conform to recognized standards or provide pathways to digital image exchange results in repeated imaging, delays in care, and additional costs to the healthcare system and patients.

We encourage CMS and other federal agencies to issue formal guidance regarding standards and exchange for dental imaging. Further, we ask that CMS encourage ASTP/ONC to include the dental imaging use case as an optional certification criterion in the Health IT Certification Program.

ePrescribing and Prescription Drug Monitoring Programs

Prescription drug monitoring is a critical issue in dentistry, given the profession's role in appropriately prescribing controlled substances, particularly opioids for acute pain management. A retrospective review of dental, medical, and pharmacy claims found that older patients with potentially inappropriate medication combinations (PIMCs) involving opioids prescribed by dentists experienced higher rates of acute care use.[6]

Dentists are required to register with their state's Prescription Drug Monitoring Programs (PDMP) when dispensing or prescribing controlled substances. Some state regulations, such as New York State Department of Health, require most prescribers to consult the Prescription Monitoring Program (PMP) Registry when writing prescriptions for Schedule II, III, and IV controlled substances. The PMP Registry provides practitioners with direct, secure access to view their patients' dispensed controlled substance prescription histories. Studies indicate that dental providers find PDMPs useful, and some reported changes in opioid prescribing behavior after reviewing prescription histories[7].

However, dental practices are often challenged by limited integration with state PDMPs. While the ADA is unaware of studies specific to dental PDMP integrations, we note that prior research has concluded that EHR integration with PDMP was associated with an increase in use of the database by physicians and a decline in patterns of opioid polypharmacy.[8] The

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[6] Zhou J, Calip GS, Rowan S, McGregor JC, Perez RI, Evans CT, Gellad WF, Suda KJ. Potentially Inappropriate Medication Combination with Opioids among Older Dental Patients: A Retrospective Review of Insurance Claims Data. Pharmacotherapy. 2020 Oct;40(10):992-1001. doi: 10.1002/phar.2452. Epub 2020 Sep 2. PMID: 32767780; PMCID: PMC8483014.

[7] McCauley JL, Gilbert GH, Cochran DL, Gordan VV, Leite RS, Fillingim RB, Brady KT; National Dental PBRN Collaborative Group. Prescription Drug Monitoring Program Use: National Dental PBRN Results. JDR Clin Trans Res. 2019 Apr;4(2):178-186. doi: 10.1177/2380084418808517. Epub 2018 Oct 29. PMID: 30931705; PMCID: PMC6900502.

[8] Hartung, D. M. Prescription Drug Monitoring Program Integration in the Electronic Health Record – Final Report. Rockville, MD: Agency for Healthcare Research and Quality; 2024. Prepared by Oregon State University under Grant No. R21 HS028119

Prescription Drug Monitoring Program Training and Technical Assistance Center (PDMP TTAC) also identifies interstate data sharing and integration with electronic record systems as a recommended best practice[9].

Leveraging the FHIR® US Prescription Drug Monitoring Program implementation guide would streamline PDMP usage, improve surveillance, and support real-world data initiatives. Thorough patient evaluation, including documentation of patient identity, medical history, diagnosis, and treatment regimen, along with prior opioid usage, can help improve overall safety and ensure safer prescription and drug monitoring. Further, a federally required technical standard for PDMP usage would ease administrative burden for providers whose catchment area includes multiple states, where providers currently default to manually checking numerous state systems to prevent prescription abuse and fraud.

Additionally, for patients with medical conditions that could lead to excessive bleeding after dental treatment, or those on medications that increase bleeding risk, dentists often choose to consult the patient's physician. Barriers to electronic medication reconciliation and prescription drug histories, increase risk of adverse drug reactions, which may then require further medical intervention[10].

We encourage the ASTP/ONC, CMS, and other federal agencies to develop a roadmap for the interstate exchange of ePrescribing and PDMP reporting and to ensure that prescription information is easily accessible to authorized entities. Specifically, we ask that CMS support integration of dental software systems into PDMP databases similar to medical systems.

Specific Comments on RFI Questions

Patients and Caregivers

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

PC-8 In your experience, what health data is readily available and valuable to patients or their caregivers or both?

PC-12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

a. Examples may include the following:
(1) Binding cost estimates for pre-defined periods.

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[9] PDMP Training and Technical Assistance Center. Interstate PDMP Access and Data Sharing Alignment. [PDF]. January 25, 2021. Supported by Grant No. 2019-PM-BX-K003 from the Bureau of Justice Assistance, U.S. Department of Justice

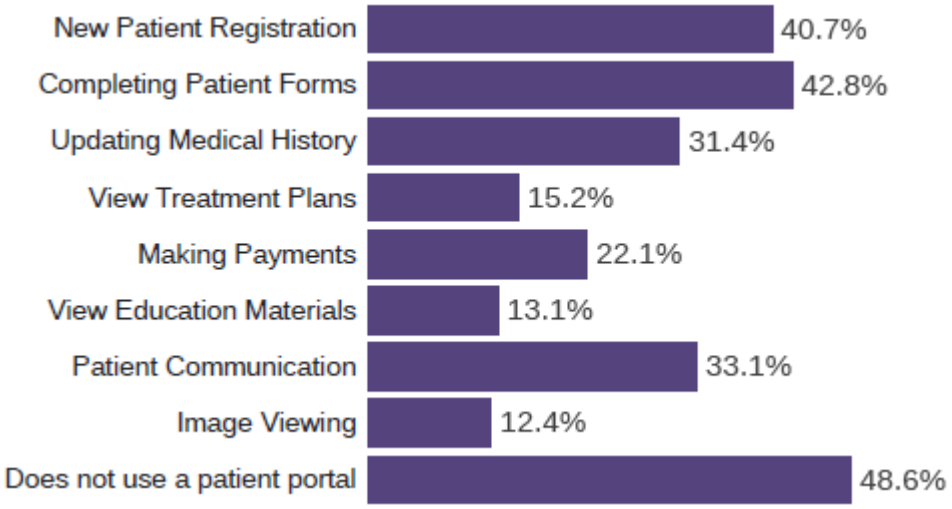
[10] American Dental Association. (n.d.). *Oral anticoagulant and antiplatelet medications and dental procedures*. ADA. https://www.ada.org/resources/ada-library/oral-health-topics/oral-anticoagulant-and-antiplatelet-medications-and-dental-procedures

(2) Viewing provider schedule availability.
(3) Using third-party apps for appointment management.
(4) Accessing patient-facing quality metrics.
(5) Finding the right provider for specific healthcare needs.

b. What use cases are possible today?
c. What should be possible in the near future?
d. What would be very valuable but may be very hard to achieve?

Dental offices often offer a patient-facing portals; however, functionality is typically limited to administrative purposes such as scheduling appointments, patient communication, pre-visit paperwork, or billing services. Access to medical records, including history, treatment plans, and imaging, is limited, and if available, typically is read-only access without the ability to share that information with third-party applications if desired by patients.

Does your practice currently use a patient portal for any of the following?



Source: ADA 2023 Dental Provider Survey on Health Information Technology

Additionally, these portals are usually restricted to the provider organization and do not integrate or connect with specialty providers or the larger healthcare ecosystem. If health information exchange access is permitted, these systems typically use proprietary data exchange, limiting records' portability.

Meanwhile, patient surveys indicate that adults are interested in receiving their dental records through a portal; more than 4 out of 5 adults stated that they wanted their dental records shared with medical providers electronically; 7 out of 10 adults wanted their patient portal to contain both medical and dental records; and 3 out of 4 adults said having access to a dental portal would improve their ability to track and understand their oral health as well as increase their trust in their dental provider[11].

[11] CareQuest Institute for Oral Health. Patient Portals: Patient Perspectives and Opportunities for Practices. Boston, MA; January 2022.

Dental records' interoperability and data accessibility for patients and caregivers in dentistry are far more limited than those for medical health information. These gaps become even clearer when medical treatment is linked to dental treatment.

We know that managing and tracking dental referrals is a challenge for providers: although 97% of dental offices refer patients for additional treatment, only 28% have electronic referral capabilities. Instead, approximately 75% continue to rely on paper-based referrals, and more than 60% of dentists are unaware of the status of their outbound referrals, trends that are consistent for both dental and non-dental referrals. Referral management is primarily handled by front office staff (92.5%) or dentists (45.4%), requiring an estimated five hours of administrative time per week.[12] Without patent tools that enable electronic access to health records, treatment plans, provider communication, and orders, patients must rely on physical paperwork and hours spent calling providers and requesting records. All of this could be done with a few clicks using standards-based application programming interfaces (APIs) that are readily available and recognized by CMS.

Despite millions of Americans seeking care in emergency departments for dental conditions each year, a lack of interoperability between hospital and dental systems results in costly, fragmented, and ineffective care. Without seamless referral pathways or shared health records, most patients receive only temporary relief—typically opioids or antibiotics—without definitive treatment. This disconnect leads to repeat visits, avoidable complications, and billions in unnecessary spending. Estimates suggest that poor communication and referral capacity between emergency and dental systems cost the U.S. healthcare system over $3.5 billion annually, driven by avoidable ED utilization, inappropriate prescribing, and missed opportunities for timely, lower-cost dental interventions. Integrating dental providers into health information exchanges and enabling real-time referrals through standards like FHIR is not just a technical fix—it is a critical policy lever to reduce waste and improve outcomes.

High-risk populations, such as the medically fragile, individuals with intellectual and developmental disabilities, and the elderly, are disproportionately impacted by the inability of dental records to integrate or exchange information between dental providers and others involved in their circle of care. Several care domains would benefit from patients having a complete picture of their dental health, for example, pediatrics, geriatric providers, palliative care, and post-acute care. App developers and patients can access key dental information from their records through standards-based APIs, allowing for innovation for remote health and home health solutions in dental care.

PC- 8c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

The ADA submitted extensive comments in response to Docket No. FDA–2025–N–0287: Exploration of Health Level Seven Fast Healthcare Interoperability Resources for Use in Study Data Created From Real-World Data (RWD) Sources for Submission to the Food and Drug Administration; Establishment of a Public Docket; Request for Comments. In summary, outstanding data quality issues, due to non-conformance to standard data models and terminology, failures to adopt broader healthcare informatics standards (e.g., DICOM®),

[12] ADA 2023 Dental Provider Survey on Health Information Technology

and the proliferation of proprietary APIs in dental technology, force the industry to rely on extensive mapping to standards-based APIs by costly third-party data brokers. This makes collecting RWD an ongoing challenge in dentistry. Collecting accurate RWD will take extensive resources (time and human capital).

The ADA recommended the FDA:

1. Encourage the use of (Fast Healthcare Interoperability Resources) FHIR® profiles and value sets as defined in the HL7 FHIR® Implementation Guide: Dental Data Exchange, Release 1 - US Realm.
2. Support accelerators for testing dental FHIR®-based information exchange.
3. Pursue the adoption of USCDI Version 5 for federal certification of health IT products to ensure that it meets the needs of the dental industry and is particularly ready to address interoperability using FHIR®.

The ADA encourages CMS to coordinate with other federal agencies to align data collection requirements and the adoption of required standards.

Providers

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)?

Ongoing technology infrastructure issues challenge dental providers. Technology used in practices may determine the possible storage formats and exchange methodologies; broadband access and high-speed internet are factors. The Federal Communication Commission has noted "there is a significant correlation between increasing broadband access and improved health outcomes, and that Internet adoption appeared to have an even greater correlation to improved health outcomes."[13] The ADA Health Policy Institute (HPI) reports that in 2023, 74.7% of dentists practiced with one or more dentists in a single location[14]. A concerted effort should be taken to ensure that data in a dental software system is accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields), and that the ability for a provider to fulfill patient record requests, regardless of format, be easily accessible and native to the software. These improvements must be sensitive to dental practice models and rural practices who may face a disproportionate financial burden when adopting and maintaining new technology.

Dental providers and their patients risk becoming a prominent example of the digital divide in healthcare. Unequal access to and use of digital tools will affect patient outcomes. Factors like geographic location and digital literacy, along with limited access to

[13] Federal Communications Commission. (n.d.). Studies and data analytics on broadband and health. FCC Health

[14] The Evolving Dental Practice Model Data Update for 2023 on Practice Size and DSO Affiliation. ADA Health Policy Institute

interoperable health data, will contribute to gaps in treatment, restrict diagnostic accuracy, and hinder the patient's ability to engage actively in care delivery. For providers, the digital divide will create barriers to participating in quality reporting and outcome measurement, increase administrative burdens, and inhibit the effective use of technologies such as AI for predictive analytics.

ADA encourages CMS to offer financial incentives and technical support to small dental practices to facilitate their participation in health information exchange and adoption of interoperable systems.

Data Exchange

PR-5 Which of the following FHIR® APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

While foundational work has been outlined in the HL7 FHIR® Implementation Guide: Dental Data Exchange, Release 1 - US Realm, vendor adoption is low. Additionally, challenges remain in harmonizing data models and value sets between medical and dental practices. Continued investments in these areas and incentives to adopt FHIR®-based APIs can aid in moving the dental industry forward in parallel with our medical counterparts.

Regarding the dental industry adoption of:

a) Patient Access API
b) Standardized API for Patient and Population Services.
c) Provider Directory API
d) Provider Access API
e) Payer-to-Payer API
f) Prior Authorization API
g) Bulk FHIR®
h) SMART on FHIR®
i) CDS Hooks (for clinical decision support integrations)

It has been difficult to gauge implementation plans and progress toward meeting regulatory deadlines. Of the required implementors, state Medicaid or CHIP programs, dental adoption is given lower priority than the medical program. Carriers that service the Medicare Advantage programs have expressed concerns that they are building roadmaps for adoption. However, due to the lack of adoption by dental software vendors, they will have limited dental end-users, creating a digital bridge to nowhere.

The CMS Interoperability and Prior Authorization Final Rule (CMS-0057-F) currently excludes stand-alone dental plans from its API requirements.

ADA recommends that CMS consider the unique needs of dental practices and expand the FHIR® API requirements to include stand-alone dental plans that participate in federally facilitated marketplace exchanges or serve as a dental benefits administrator on behalf of a Medicare Advantage plan. This would establish a

clear and unanimous path to interoperability and ensure seamless data exchange at all levels.

PR-6. Is TEFCA currently helping to advance provider access to health information?

a. Please provide specific examples.

b. What changes would you suggest?
c. What other options are available outside of TEFCA?

d. Are there redundant standards, protocols or channels or both that could be consolidated?

The ADA supports efforts to create a framework for trusted exchange; however, given the complexity of dental data and limited participation from dental software in implementation, dental providers' experience accessing and using TEFCA, QHINs, or other health information exchanges is minimal. Several state legislatures have mandated participation in the state-based data exchange endeavors, however, it is worth noting that experience has shown that either due to the lack of maturity of dental interoperability or financial challenges, this participation is often qualified by provider registration and use of a minimal viable option such as maintaining a direct secure email to receive notifications.[15] The ADA is unaware of any specific datasets regarding dental participation, ability to meet state mandates for health information exchange, or involvement in TEFCA.

Payers

PA-2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

Dental spending on administrative processes increased in 2023 as dental providers and staff managed a higher volume of manual and electronic tasks. The 2024 edition of the CAQH Index is the only health care industry benchmarking tool that tracks and reports progress on the adoption of electronic administrative transactions and the potential savings associated with transitioning from manual processes. It is prepared by the Council for Affordable Quality Healthcare, the health care industry alliance commonly known as CAQH. The 2024 report consists of data from 2023. Eligibility and benefit verification accounted for the largest increase in spending, rising 15% to $2.1 billion. Verification checks should ideally be conducted fully electronically; however, due to a lack of complete and accurate data, providers often rely on conducting these checks through portals. The portals, maintained by insurance companies, vary in requirements and formats, which complicates verification.[16]

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[15] Connecticut General Assembly. (n.d.). Conn. Gen. Stat. § 17b-59e (2025). In Chapter 319o: Department of Social Services.
[16] Council for Affordable Quality Healthcare. (2025). *The CAQH Index 2024: From Transactions to Trust: Building better care through healthcare automation* [Annual industry report]. Retrieved from https://www.caqh.org/insights/caqh-index-report

The ADA encourages efforts to educate, engage, and incentivize dental business partners to adopt FHIR® APIs, which supplement the mandated transactions to support administrative simplification and burden reduction. HL7 projects such as the DaVinci Burden Reduction implementation guides may offer new opportunities for all types of benefit administration; however, these have yet to be tested and validated by the dental community.

The ADA recommends that CMS pursue opportunities to promote, fund, and engage in testing of FHIR® APIs to reduce the administrative burden for dental use cases.

Technology Vendors, Data Providers, and Networks

TD. 4 How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

The ADA recommends several actions that would encourage the use of open, standards-based, and publicly available APIs:

a. Education—Industry-specific education, materials, and communications highlighting the benefits and return on investment for dental practices, vendors, and data trading partners. Educate patients regarding their data rights, specific to those CMS program requirements, and require that plans communicate with patients about leveraging Patient Access APIs.
b. Transparency and Compliance – Most dental record systems are not certified under the ONC Health IT Certification Program (Certification Program), nor have vendors self-selected to attest to meeting technological requirements adopted by the U.S. Department of Health and Human Services. Encouraging public-facing transparency of dental vendor functionality and conformance may improve market demand for compliance and assure software purchasers that these products will meet expected functionality.
c. Foster Innovation—Create pilot projects and public-private partnerships that test innovative, standards-based dental data exchange solutions. Tools specifically tailored to oral health needs, including patient care coordination and integration with medical disease management, can help move the dental industry forward, paralleling medical technology. Facilitate collaboration across stakeholders, including payers, providers, vendors, patients, and standards development organizations, to align priorities and accelerate innovation.
d. Demand Accountability--Dental data relies on domain-specific clinical data structures, as identified in ANSI/ADA Standard No.1084 - Reference Core Data set for communication and adopted in the HL7 FHIR® Implementation Guide: Dental Data Exchange, Release 1 - US Realm. When applied to dental stakeholders, federally mandated or recommended FHIR® IGs should harmonize with the above standards.

TD-5. How could a nationwide provider directory of FHIR® endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

The ADA supports efforts to establish a nationwide provider directory for FHIR® endpoints, as this will aid ongoing initiatives to reduce administrative burdens and facilitate information sharing among providers, patients, and other stakeholders. The ADA encourages CMS to consider how provider directories and FHIR® endpoint directories can be harmonized to ensure accurate and up-to-date information about contracted provider networks while streamlining access to this information. Patients often struggle to find providers who not only accept their insurance benefits but also accept new patients. Dental payer-based provider directory APIs frequently contain outdated or incomplete data, resulting in “ghost” networks.

The ADA has collaborated with CAQH ProView to offer a singular provider information source of truth for credentialing, surpassing 147,598 dentists using the system nationally, and over 312 entities that use the repository as consumers. CAQH was formed by health plans who came together nearly 25 years ago with the desire to make healthcare work better. The information in this repository supports ADA efforts to promote industry solutions that reduce administrative burden, allowing practices to spend more time on clinical care and less on paperwork. **Systems that maintain provider data for purposes such as credentialing can easily be adapted to support use cases such as directories without the need for duplicative systems.** Despite these efforts, there continues to be a proliferation of directory solutions, often required by mandate for participation in state Medicaid programs. The need to maintain accurate information in a provider directory is placed onto the dental practice. Failure to implement a single source of information leads to significant duplication of effort by providers and practices and creates data inconsistencies. Furthermore, many dentists opt not to enroll as providers for Medicare due to additional administrative burdens in enrollment and credentialing through the Provider, Enrollment, Chain, and Ownership System (PECOS). While it is a centralized system, it does not have other mechanisms of interoperability with other credentialing systems that collect similar information. This ultimately duplicates efforts in program enrollment and credentialing already being done in the private sector in a more efficient, cost-effective manner.

The ADA requests CMS promote the centralization and standardization of provider data collection, with a program like CAQH, that allow providers to choose a single-entry point for information, enabling end users of provider information to leverage FHIR® APIs to receive timely, accurate data nationally.

TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

a. What are the primary obstacles to this?

b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

The ADA, an ANSI-Accredited Standards Developer, sets standards for nearly every tool of dentistry, ensuring their safety, reliability, and efficacy for dentists and the public. The ADA Standards Program brings over four hundred volunteer subject matter experts to participate in the development of national and international dental standards. The ADA Standards Program is leading efforts to develop ADA standards on data content such that any current or future exchange mechanisms such as emerging API technology can support exchange of

required data to serve dental patients[17]. Notably, ANSI/ADA Standard No. 1000 – Standard Clinical Data Architecture (2010; reaffirmed 2020) which defines a logical data model for clinical dental information exchange—universal across health professions, locales, and data structures; ADA Standard No. 1113 for Dentistry – Contents for Dental Procedure-Level Data Required for Eligibility and Benefit Responses and ANSI/ADA Standard No.1079 for Standard Content of Electronic Attachments for Dental Claims defines the data content to be communicated between providers and payers for standardized and seamless communication improving the administrative efficiency, and ANSI/ADA Standard No. 1084 — Core Set of Reference Data for Communication Among Dental Information Systems: 2019, which established the Reference Core Data Set (RCDS) for the structured electronic data elements to support key information exchange among and between dental and other health care settings.

ADA believes that USCDI should be perceived as the regulatory ground floor for accessible data and can continue to serve as the benchmark for certification programs. When identifying certification criteria under the ASTP/ONC Health IT Certification Program, it could be beneficial to identify domain-specific modules and attestation criteria appropriate for dental developers, as these may differ from broader interest developers. The ADA would be available to assist in developing such criteria.

Leveraging API access is a step in the right direction for opening electronic health information to new opportunities and advancements, especially when thoughtfully utilizing large language models (LLM) and AI tools that may be able to extract and validate previously inaccessible information. Precision medicine, clinical decision support, and innovation will all benefit from opening access to the entirety of a patient's electronic health information. Research institutions such as the Regenstrief Institute are using automated computer algorithms to identify disease early, track disease progression, and identify opportunities for improved outcomes, all made possible by using information that is not easily accessible (chart notes)[18].

Furthermore, enabling full EHI access through APIs will support efforts to create and maintain longitudinal dental records, including the ongoing documentation of dental health over time. A lack of longitudinal dental records may compromise patient care, increase the risk of medical errors, lead to legal and ethical issues, and negatively impact dental practice efficiency and patient care. When dental records include complete and comprehensive longitudinal data, both patients and providers experience significant benefits. This can empower patients to make informed decisions about their care delivery and improve outcomes and continuity of care.

The ADA also recognizes the importance of the HIPAA Minimum Necessary Rule, which requires that covered entities and business associates make reasonable efforts to limit the use, disclosure, and request of protected health information (PHI) to the minimum necessary to accomplish the intended purpose. This means only disclosing the minimum amount of PHI needed to achieve a specific goal, like treatment, payment, or operations.

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[17] American Dental Association. (n.d.). Informatics standards, technical specifications and technical reports. ADA Standards Committee on Dental Informatics.

[18] Regenstrief Institute. (n.d.). Dental Health Informatics

Before leveraging EHI via APIs, the industry must ensure that appropriate, mature safeguards are in place for privacy, security, and confidentiality.

The ADA encourages CMS and its federal partners to engage patients, providers, and industry partners to establish operational best practices for identity management, disclosures, and data stewardship. As data exchange expectations expand, domain-specific information should rely on key stakeholders like the ANSI Accredited ADA Standards Program to ensure that data exchange requirements are appropriate and feasible for the specific use case.

Thank you for considering our comments on this critical issue. We welcome the opportunity to discuss these concerns and recommendations with you or your staff in more detail. If you have any questions or need additional information, please do not hesitate to contact Rebekah Fiehn, Director, Dental Benefits, Coding, and Data Exchange, at fiehnr@ada.org.

Sincerely,

Brett Kessler, D.D.S.
President

Elizabeth Shapiro, D.D.S., J.D.
Interim Executive Director